

COVERAGE & CONTINUITY

Professional Will & Practice Succession Plan

Name a professional executor, protect your patients and records, and give your family a clear plan if you die or cannot practice.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Will + inventory + acceptance

VERSION

1.0 · Sept 2026

TIME TO ADAPT

1–2 hours

WHAT IT IS

A written plan that names a colleague (your “professional executor”) to notify patients, secure and transfer records, and close your practice if you die or become unable to work.

USE IT WHEN

Always. It matters most for solo practitioners, but contractors in group practices need one too.

CUSTOMIZE FIRST

Your state’s rules on records custody, notification and estate law; your liability policy’s tail-coverage terms; who your executor is.

Why you need one

If you die or become unable to work without warning, patients lose their clinician overnight, records may be inaccessible or unprotected, and your family is left guessing. The APA Ethics Code expects planning for exactly this (Standards 3.12, 6.02(c) and 10.09). Surveys and practitioner guides consistently find that most clinicians agree they need a professional will and far fewer have one. The hardest step is starting.

IT IS NOT A LEGAL WILL

A professional will is normally an instruction document. Unless your executor signs it and an attorney has checked it against your state’s law on records custody, notification duties and estates, it may express your wishes without binding anyone. Ask your executor to sign the acceptance page (Part C), give a copy to your personal executor, and have an attorney review it.

Step 0: Add one paragraph to your informed consent

Patients should hear about your executor from you, in advance, rather than from a stranger. Suggested wording:

I have named [Name], a licensed [psychologist / psychiatrist], as my professional executor. If I become seriously ill, incapacitated or die, [Name] may access my records to notify you, help you obtain your records, offer interim support if you need it, and help you find another qualified clinician.

Part A: Professional Will

1. Purpose and status

I, [full name, credential] (License [number], [state]), make this Professional Will to guide the care of my patients and the closure of my practice, [practice name], if I die or cannot practice. It does not replace my personal will or estate documents. My personal executor is [name and contact].

2. Professional executor and alternate

Professional executor: name and credential

License number and state

Phone

Secure email

Relationship to me

Alternate executor: name and credential

License number and state

Phone

Secure email

Relationship to me

My executor is a licensed colleague I trust, who is not a member of my family, is competent to handle confidential records, and has agreed to serve by signing Part C. Mutual arrangements, where two colleagues serve as each other's executor, work well and share the load.

3. When this Will takes effect

- My death.
- Incapacity: I am unable to practice for more than [14] consecutive days because of illness or injury, as confirmed by [my physician / my personal executor / any two of the following: physician, spouse or partner, executor].
- Unreachable: I cannot be contacted for [3] business days and my emergency contact cannot locate me.
- Any other sudden loss of the ability to practice (for example, license suspension) where patient transition is needed.
- If I recover, this Will's authority ends when I notify my executor in writing or my physician clears me to return.

4. Authority given to my professional executor

My executor may: access my practice phone, email, schedule, EHR, telehealth platform and files; contact patients; notify colleagues, supervisees, consultation-group members, payers, my landlord and my liability carrier; arrange transfer of care; respond to record requests; consult my attorney and carrier; engage a records vendor; and be reimbursed for reasonable costs and compensated at [rate or "no charge"].

5. Notifying patients

- Timing: within [2] business days of activation, in this order: patients in recent crisis or at higher risk, then weekly patients, then all others.

- Method: a phone call for higher-risk patients; a letter or secure message for others.
- Content: only that I am unable to continue, how to get records, who to call, and crisis resources (911, and 988 by call, text or chat). No medical detail beyond what my family has authorized.
- Only my executor contacts patients. Patient identities are not disclosed to anyone except the patient and those legally entitled.

Dear [Patient first name], I am writing on behalf of Dr. [Name], with whom you have been working. Dr. [Name] is [unable to continue seeing patients / has died], and I have been asked to help make sure your care continues safely. I am sorry to share this news in a letter. You can reach me at [phone] to arrange a copy of your records, to talk about how you are doing, or to be connected with another clinician. If you are in crisis, call 911, or call or text 988. Sincerely, [Executor name and credential]

6. Continuing care

My executor may offer interim support if it is clinically appropriate and within their competence, and will otherwise refer patients to the colleagues listed in [referral list location]. For patients I prescribe for, my executor will arrange bridging prescriptions and medication management through [named prescriber].

7. Records

- Where they are: [EHR name; paper storage location; backups]. Access credentials are stored in [password manager with emergency access / with my attorney / safe deposit box]. Never write passwords in this document.
- Retention: [state-required period]. The APA Record Keeping Guidelines suggest considering seven years after the last service for adults, and three years after the age of majority for minors, whichever is later, unless state law or contracts require longer.
- Custodian after closure: [named person or records-custodian service].
- Requests: patients' access requests are honored under HIPAA and state law. Subpoenas and legal demands go to [attorney] before anything is released.
- Psychotherapy notes (HIPAA definition): [secure / destroy / transfer only with patient authorization].
- Disposal: secure destruction with a certificate, after the retention period ends.

8. Business and financial matters

- Outstanding receivables handled through [billor or executor]. Prepayments refunded promptly. Recurring card charges stopped.
- Cancel or transfer: lease, phone, domain, website, telehealth and EHR subscriptions. Notify payers and clearinghouses.
- Professional liability: notify my carrier and ask about extended reporting ("tail") coverage. Claims-made policies usually need it.
- Final accounting and taxes with [accountant].

9. Communications with others

My executor will notify: reciprocal-coverage colleagues, consultation-group members, supervisors and supervisees (including any board-required supervision paperwork), referral partners, my licensing board if required, and will post a brief message on my website and voicemail directing patients to [contact].

10. Review

I will review this Will every [12] months and after any major change in my practice, health or family. Last reviewed: [date].

Clinician signature

Date

Professional executor acceptance (see Part C)

Date

Alternate executor

Date

Part B: Practice continuity inventory

Tell your executor where things are and who holds access. Do not write passwords here. Keep the master copy with your executor and your attorney.

Item	Details and where to find it	Access held by
EHR / practice management	[system, account name]	[]
Telehealth platform	[]	[]
Practice phone and voicemail	[number, provider]	[]
Practice email and secure messaging	[]	[]
Paper records and keys	[location]	[]
Backups and drives	[location, encryption key holder]	[]
Password manager and emergency access	[]	[]
Billing service, clearinghouse, merchant account	[]	[]
Payer portals and CAQH	[]	[]
Professional liability, cyber and business insurance	[carrier, policy numbers]	[]
Attorney, accountant, financial contact	[names, phones]	[]
Landlord and lease	[]	[]
Website and domain registrar	[]	[]
Consultation groups, supervisors, coverage colleagues	[names, contacts]	[]

Part C: Executor acceptance and emergency card

I, [executor name, credential] (License [number], [state]), accept designation as Professional Executor for [clinician name]. I understand this role involves confidential patient information and agree to act in patients' best interests, follow applicable law, keep information confidential, and consult counsel when legal questions arise. I may decline or resign by notifying [clinician] and the alternate executor in writing.

Executor signature

Date

Wallet card text

In an emergency or if I am unable to respond, please contact my professional executor, [Name], at [phone], and my personal contact, [Name], at [phone]. [Your name, credential]

Practice notes

- Ask a colleague in person. Framing it as a mutual arrangement makes the request easier.
- Review it every year on the same day you renew your license or liability policy (see PA-19).
- Tell your family where the executor's details and Part B are kept.
- Some states have specific rules on notice or records custody after a clinician's death. Ask your board or attorney.

IN PSYALLIANCE

Your Partners tier is a natural place to find an executor and reciprocal-coverage colleague. Keep your license expiry and insurance details current in Credentials so your executor can find them.

Sources and further reading

- APA Ethics Code, Standards 3.12, 6.02(c) and 10.09 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- APA Record Keeping Guidelines ([apa.org/practice/guidelines/record-keeping](https://www.apa.org/practice/guidelines/record-keeping)).
- Society for Psychotherapy, "Psychotherapist Professional Wills: Easy to Avoid, Crucial to Address" (2025).
- K. Pope, "Therapist's Guide to Preparing a Professional Will" (kspope.com).

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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